Prodigy Anesthesia Quick Review

Cerebral Palsy

What is cerebral palsy?	Cerebral palsy is not a specific disease, but a varied set of symptoms that occur due to lesions of the brain that occur during early stages of development. *Hines RL, Marschall KE. Anesthesia & Co-Existing Diseases. 6th ed. Philadelphia, PA: Elsevier-Saunders; 2012: 604-605.*
How is cerebral palsy classified?	Cerebral palsy is classified according to the extremities involved (quadriplegia, monoplegia, etc.) and the type of neurologic dysfunction (hypotonic, spastic, athetotic, etc.). *Hines RL, Marschall KE. Anesthesia & Co-Existing Diseases. 6th ed. Philadelphia, PA: Elsevier-Saunders; 2012: 604-605.*
What is the most common symptom of cerebral palsy? What other conditions may accompany cerebral palsy?	Skeletal muscle spasticity is the most common symptom of cerebral palsy. Other symptoms that may appear are seizures, choreoathetosis, dystonia, mental retardation, cerebellar ataxia, and speech disorders. *Hines RL, Marschall KE. Anesthesia & Co-Existing Diseases. 6th ed. Philadelphia, PA: Elsevier-Saunders; 2012: 604-605.*
What medications are used in the treatment of cerebral palsy?	Medical treatment is often aimed at the relief of muscle spasticity and include medicines such as dantrolene, Botox injections, and baclofen. *Hines RL, Marschall KE. Anesthesia & Co-Existing Diseases. 6th ed. Philadelphia, PA: Elsevier-Saunders; 2012: 605.*
What surgeries are patients with cerebral palsy likely to undergo?	Children with cerebral palsy often undergo elective surgery to correct musculoskeletal disorders. Common surgeries include Achilles' tendon lengthening, osteotomy of the femur, iliopsoas release, and scoliosis correction. Gastric reflux is often severe enough that surgery to correct it is necessary. *Hines RL, Marschall KE. Anesthesia & Co-Existing Diseases. 6th ed. Philadelphia, PA: Elsevier-Saunders; 2012: 605.*
What are the anesthetic considerations for the patient with cerebral palsy?	They have an increased incidence of reflux and weak pharyngeal and laryngeal muscles. They are susceptible to hypothermia and require close temperature monitoring. Tracheal extubation may need to be delayed until the patient is fully awake and airway reflexes return. MAC is generally decreased and emergence times are longer. *Hines RL, Marschall KE. Anesthesia & Co-Existing Diseases. 6th ed. Philadelphia, PA: Elsevier-Saunders; 2012: 605.*
Many patients with cerebral palsy take baclofen. Why is it so important to ensure that baclofen is continued throughout the perioperative period?	Abrupt discontinuation of baclofen can result in withdrawal symptoms such as itching, confusion, hallucinations, and seizures. *Hines RL, Marschall KE. Anesthesia & Co-Existing Diseases. 6th ed. Philadelphia, PA: Elsevier-Saunders; 2012: 605.*

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Is succinylcholine contraindicated in patients with cerebral palsy?	Even though they may suffer muscle weakness and spasticity, patients with cerebral palsy do not have an exaggerated hyperkalemic response to succinylcholine, nor are they more susceptible to malignant hyperthermia. *Hines RL, Marschall KE. Anesthesia & Co-Existing Diseases. 6th ed. Philadelphia, PA: Elsevier-Saunders; 2012: 605.*
How do patients with cerebral palsy respond to nondepolarizing muscle relaxants?	The use of antiseizure medications is common in these patients and they may display a subsequent resistance to nondepolarizing muscle relaxants. *Hines RL, Marschall KE. Anesthesia & Co-Existing Diseases. 6th ed. Philadelphia, PA: Elsevier-Saunders; 2012: 605.*

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